Which treatment this guide covers This guide covers lorlatinib alone for ALK-positive advanced non-small-cell lung cancer. The available Irish NCCP v3 document lists use after particular earlier ALK medicines. The current European licence also includes people who have not had an ALK inhibitor. The team must confirm your treatment setting and the current Irish protocol and access arrangements before prescribing; the source versions currently disagree.

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Lorlatinib therapy

Lorlatinib is taken once daily continuously, with no planned rest week. Treatment continues while it is helping and side effects remain acceptable; there is no fixed number of treatment cycles.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Raised cholesterol and triglycerides; pancreatic inflammation may occur

☐ Numbness, tingling or nerve symptoms

☐ Heart conduction problems, fainting and reduced heart function

☐ Lung inflammation that can be serious

☐ Harm to an unborn baby, contraception interactions and possible impaired male fertility

☐ Changes in memory, thinking, mood, speech or perception, including serious psychiatric effects

☐ Swelling, weight gain and fatigue

☐ High blood pressure and raised blood sugar

☐ Diarrhoea, nausea and skin effects

Discuss the individual treatment setting, likely benefit and uncertainty, alternatives including not proceeding, brain/mood, metabolic, cardiac and lung risks, possible lasting effects, daily tablets, interactions, pregnancy/fertility and the monitoring/contact plan. Confirm the current Irish source/access discrepancy with the clinical team. This aid supports an ongoing consent conversation.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before starting: ALK testing, blood count, kidney and liver tests, clotting tests, cholesterol and triglycerides, glucose, pancreatic enzymes, blood pressure and an ECG. Lipids are checked at weeks 2, 4 and 8, then as needed. Blood count, kidney/liver tests, ECG and pancreatic enzymes are normally checked monthly. Blood pressure is checked after two weeks and at least monthly; glucose as needed. Scans follow your oncology plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.24 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending